A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

14 May 2026

Mr James Keck
Melbourne Colorectal Specialists

Dear Mr Keck,

Re: Mrs Alison Tanner
DOB: 19/07/1950 Phone: 0448819745
Address: 3 Hotham Court, MONT ALBERT, VIC, 3127
Email: tgifriday@netspace.net.au

Summary

- **Bloating and abdominal discomfort likely secondary to abdominophrenic dyssynergia.**
- **Symptoms exacerbated by a significant period of psychosocial stress and high alcohol intake.**
- **Commenced on low-dose escitalopram for neuromodulation and visceral hypersensitivity.**
- **Recommended implementation of partially hydrolysed guar gum (PHGG), kiwifruit, and use of a footstool to optimise defaecation.**
- **Advised significant reduction in alcohol consumption to assist with symptom management.**
- **No action required by you.**

I had the pleasure of conducting an in-person review with Alison and her husband Bruce, thank you for referring her.

Symptoms and history of presenting complaint

Alison describes a primary symptom of significant bloating and abdominal discomfort. She describes the bloating as a physical distension where she feels "six months pregnant," often occurring in the afternoon following lunch. She notes the abdomen feels like a "tight little drum" and does not always resolve with the passage of flatus or a bowel movement. She also experiences lower abdominal pain and an occasional "anal sensation."

These symptoms developed approximately three to four weeks following an emergency surgical procedure at the Epworth in late 2024. Prior to this, she experienced a sudden onset of severe abdominal pain and vomiting (described as a "tsunami" of pain), which led to a CT scan identifying a redundant bowel. While the pain settled spontaneously, she underwent surgery with Mr Niaz Naqash. Interestingly, she was asymptomatic between the acute pain event and the surgery, and remained well for several weeks postoperatively until the current bloating complex began around Christmas.

Her bowel habit occurs every one to two days. The consistency is variable, ranging from normal stools to "ribbon-like" or small "pebbles." She reports occasional explosive movements and has recently noted the need to strain, which is a new development for her. She occasionally uses suppositories to assist with evacuation. She denies any haematochezia or mucus in the stool.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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Diet

Alison's diet is relatively consistent. She is not a "big breakfast person," often consuming only a piece of fruit, such as mandarins. Her caffeine intake is moderate, consisting of three to four coffees with milk daily. She has recently switched from sugar-free cordial to a sugar-based version. Her alcohol intake is significant, currently consuming one bottle of white wine per night on a daily basis. She notes she has a "sweet tooth" and her meals typically consist of salads or soup for lunch, and a meat and vegetable stir-fry for dinner.

Past medical history and medications

Alison's medical history is significant for a Protein S deficiency and previous deep vein thromboses (DVTs). She has also undergone a right hip replacement, orthopedic surgery on her right elbow, and a big toe procedure which was complicated by a protracted infection requiring nine months of antibiotics.

Her current medications include:

- Xarelto (anticoagulant)
- Coveram (for hypertension)
- Vitamin D
- Occasional Nexium or Mylanta for mild reflux symptoms.

Treatments trialled before and investigations performed

She has previously trialled Buscopan for abdominal spasms and probiotics, neither of which provided significant relief. Investigations to date include two CT scans, an MRI (reported as anatomically normal), and a transit study. She also had a normal colonoscopy performed by Mr Nakash. A gastroscopy performed some years ago showed mild oesophagitis.

Social history, family history

Alison is a retired nurse who recently ceased work due to the impact of her gastrointestinal symptoms. She lives with her partner, Bruce. She has three biological children (all vaginal deliveries, with an episiotomy for the first) and three step-children. The last two years have been significantly stressful, particularly regarding her daughter's marital issues and her own health concerns. There is no family history of inflammatory bowel disease, coeliac disease, or gastrointestinal malignancy. She is a lifelong non-smoker.

Examination

On examination, she appeared comfortable but anxious. Her hands and oral cavity were unremarkable. Abdominal examination revealed a soft, non-tender abdomen. There were no palpable masses or stigmata of chronic liver disease.

Impression and plan

My impression is that Alison is suffering from abdominophrenic dyssynergia. This is a functional coordination disorder where a trigger—likely the previous surgical intervention and bowel irritation—activates a reflex that causes the diaphragm to descend and the abdominal wall to protrude. This system has likely remained "switched on" due to the concurrent period of high psychosocial stress and visceral hypersensitivity. I do not believe further mechanical or surgical intervention is warranted, as her imaging and endoscopic workups

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have been reassuring.

Our plan is as follows:

1. **Neuromodulation:** I have prescribed a low dose of escitalopram (10mg). While primarily an SSRI, at this dose it acts as a neuromodulator to down-regulate the nervous system's sensitivity in the gut. We discussed that it may take six to eight weeks to see an effect.
2. **Bowel Regulation:** I have recommended a fibre supplement in the form of partially hydrolysed guar gum (PHGG), which is generally well-tolerated without increasing gas. I have also suggested incorporating two kiwifruit daily.
3. **Defaecation Posture:** I have advised the use of a footstool (such as a 'Squatty Potty') to optimise the anorectal angle and ease evacuation.
4. **Lifestyle:** We had a frank discussion regarding her alcohol intake. I have strongly advised her to reduce this significantly, aiming for at least several alcohol-free nights per week and limiting consumption to one to two glasses when she does drink.
5. **Follow-up:** I will review Alison via telehealth in approximately eight weeks to assess her response to these interventions.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Caroline Johnson

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